Earn the first 500. Learn what makes them stay.

Don’t manufacture a menopause influencer. Build a useful moment women choose to pass along.

Our bet: a private, non-diagnostic “Pattern Mirror” turns a confusing set of changes into a clear reflection and a better question to take into the next step.

Why test it: a 2025 survey paper cites a prior UK study in which 63% of surveyed perimenopausal women sought information from friends. That suggests a social discovery route—not proof of AxisX demand. [1]

500

ACTIVATED COHORT

Working definition: app install + onboarding complete.

60%

D7 RETENTION TARGET

300 of 500 activated users return on day 7.

$10

PILOT ALL-IN / ACTIVATION

$5,000 ÷ 500. Not a paid CAC; media excluded.

834

IF GOAL MEANS 500 D7

At 60% D7, 834 activated users yield ~500 retained.

Start with the woman asking: “Could these changes be connected?”

BEACHHEAD HYPOTHESIS—NOT A FACT

A midlife woman with several new, disruptive changes and no satisfying way to make sense of them.

Test ages 38–50 as a recruitment hypothesis, not a targeting rule. Lead with sleep, mood, energy or cycle changes; let users self-identify. First geography/language remains a brief question. Exclude diagnosis-seeking claims, pregnancy/postpartum care, crisis needs and conditions AxisX cannot safely support.

01

Landing page

Clear promise: reflect on patterns and prepare questions. No claims that AI detects hormones or what a clinician missed.

02

Optional quiz

Short, skippable needs screener. Avoid collecting detailed health data until necessity, consent and retention are clear.

03

Application

Explain closed-test fit, eligibility, privacy and invite timing. Separate marketing consent from app access.

04

Invite → activation

Instrument invite opened, install, onboarding complete and D7 return as separate events; define D7 window before launch.

Evidence: survey findings support an information gap, but not this specific segment or conversion hypothesis. [1]

Make the useful moment the campaign

The stronger alternative to five synthetic characters: a real utility loop. AI can tailor reflection and adapt education; women supply the lived experience, with consent and human review.

01

Reflect

Quiz yields a private, non-diagnostic summary: “what you noticed,” not “what you have.”

02

Understand

AI generates a plain-language prompt and questions for a trusted professional. Reviewed source library; uncertainty visible.

03

Choose to share

One-tap share card contains education or an invitation only—never symptoms or inferred health status by default.

04

Invite a circle

Referral link grants access priority or a useful guide; no reward tied to symptom disclosure or medical decisions.

05

Return with value

Opt-in weekly reflection, saved questions and a user-set check-in create a reason for D7 use.

Loyalty: progress, not points

Personal milestones, saved insights, “bring this to my appointment” exports, and a small moderated community.

Referral: permission, not pressure

Reward the inviter only after invitee independently opts in; make sharing revocable and easy to skip.

Guardrails

No fake testimonials, synthetic lived experience, sensitive-health retargeting or diagnostic claims. Disclose paid endorsements clearly. [2]

[2] FTC influencer disclosures guidance; confirm applicable jurisdictions and health-data rules before launch.

$5k buys a decision—not vanity reach

BUDGET CEILING • TOOLS, TOKENS, CONTENT ONLY

Use	USD	Test / deliverable
Funnel + analytics tools	$900	Landing, quiz, invite tracking
AI tokens + safety review	$700	Pattern reflection prototype; reviewed prompts
Creative production	$1,200	Reusable explainers, share card, 3 narrative formats
Partner/community kits	$900	Facilitator materials, newsletter/clinic outreach
Research + user sessions	$800	Recruiting, interviews, usability checks*
Contingency	$500	Privacy/legal/tooling adjustments
Total	$5,000	No paid media

*Only if incentives are permitted and included; exclude team time and app development pending organizer clarification.

ILLUSTRATIVE VOLUME MATH—NOT A FORECAST

~4,200

qualified landing visits × 40% quiz completion = 1,680 completions

~840

applications at 50% of completions

~500

activated at 60% invite-to-onboard; requires adequate test slots and invitations

~300

D7 retained if cohort retention reaches 60%

If “500 users” means 500 D7-retained, target ~834 activated at 60% retention. This is a material scope change; validate the denominator before promising volume.

Funnel rates are planning assumptions to instrument and replace with observed cohort data.

$50k is a gated channel experiment—not a scaled-up pilot

BUDGET • RELEASE IN TRANCHES

Paid search, high-intent themes — $14k

Test message/keyword cohorts; no health-condition audience lists or inferred sensitive targeting.

Contextual sponsorships — $10k

Women's health newsletters, podcasts and relevant contextual placements; holdout/referral codes.

Human creator partnerships — $8k

Small creator tests based on genuine experience or education; clear ad disclosure; never scripts posing as personal outcomes.

Creative iteration — $7k

Variations of utility-led hooks, educator Q&A and quiz; optimize qualified activation, not clicks.

Measurement, privacy + reserve — $11k

Attribution, cohort analysis, compliance review and contingency; exact vendor scope after first-phase data.

$50,000 total

DECISION ARCHITECTURE & UNIT ECONOMICS

Gate 0 • before media

Confirm pricing/LTV hypothesis, privacy review, test slots, baseline onboarding and event definitions.

Gate 1 • $8k test

Test channels/creative separately. Wait for D7 cohorts; do not reallocate on CTR alone.

Gate 2 • up to $17k

Release only if a channel's cost per activated user is below the provisional ceiling and D7 is not materially worse than organic.

Gate 3 • remaining $25k

Scale proven combinations; pause if marginal CAC rises or retention falls. Cap spend by cohort evidence, not budget availability.

Illustration, not forecast: at $30k acquisition spend and $10 media CAC/activated, 3,000 activations; at 60% D7, 1,800 retained. Fully loaded $50k CAC = $16.67/activation or $27.78/D7 user. Paid ceiling must ultimately derive from verified paid conversion × gross margin × retention, still unknown.

Ten-times-volume question: does marginal CAC and D7 quality survive channel saturation? Prove before releasing tranches.

Measure the first 500 so the next $50k is evidence-led

EVERY USER, BY SOURCE + MESSAGE + COHORT

Funnel

Landing visits → quiz starts/completes → applications → invites → installs → onboarding completion. Timestamp and campaign/source IDs.

Retention & value

D1/D7/D30 return, meaningful action definition, feature use, opt-outs, qualitative “why stayed/left,” willingness-to-pay at randomized price points.

Growth loop & cost

Share artifact use, invitations sent, invite acceptance, referred activation, referral coefficient, fully loaded pilot cost and channel-level marginal CAC.

Trust & safety

Consent rates, deletion requests, complaint/incorrect-output reports, content flags; minimize and segregate health data from ad systems.

Go to paid test only if D7 approaches the 60% goal, onboarding and source attribution are reliable, and paid conversion economics can be bounded. 60% is AxisX’s goal—not an external benchmark.

QUESTIONS FOR THE CASE TEAM

1. Who is the first user by age, needs, geography/language, and what is out of scope?
2. What product features work today; what is AI-powered and what claims are cleared?
3. What do closed-test activation, D7, feedback and willingness-to-pay already show?
4. Does “500” mean activated or D7-retained, and by when?
5. What are invite mechanism, platforms, test capacity, eligibility and app-store constraints?
6. What price, paid feature, trial and conversion event should be tested?
7. What health-data, clinical, testimonial, influencer and ad-targeting guardrails apply?
8. Which founders, clinicians, partners, owned audiences, brand and analytics can we use?
9. Which costs count in $5k; are incentives, legal review and team time in-kind?
10. What duration, market, CAC ceiling and release/stop thresholds apply to $50k?
11. Which loyalty/referral benefits are allowed and what would feel coercive?
12. Which competitors and substitutes matter most?

[1] Survey evidence is directional; validate segment and demand with AxisX cohort data. [2] FTC disclosures guidance.